

Medical Legal Report

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1. Demographics

a.

Sure, here are the itemized names and addresses extracted from the defense attorney, applicant attorney letter, and the insurance company from the document: 1. ****Defense Attorney:**** - Name: Not explicitly mentioned. - Firm: Morgan and Leahy LLP - Address: 30101 Agoura Court Suite 200, Agoura Hills, CA 91301. 2. ****Applicant Attorney:**** - Name: Emily Mehr, Esq. - Firm: Mehr Associates Irvine - Address: 17500 Red Hill Ave Suite 210, Irvine, CA 92614. 3. ****Insurance Company:**** - Name: Sedgwick CMS - Address: PO BOX 14188, Lexington, KY 40512. These details are directly extracted from the provided documents. If you need further clarification on specific names or addresses, please let me know!

b.

Here is the detailed information extracted from the provided document: 1. ****Claimant Name:**** Tiu, Michelle E. 2. ****Claimant Date of Birth:**** 10/31/1976 3. ****Claimant Employer:**** KP SCAL 4. ****Claimant Occupation:**** Optometrist 5. ****Claimant Date of Injury:**** 5/28/2024 6. ****Claim number:**** 2024050001029 7. ****WCAB Number:**** ADJ19391023. Please let me know if you need further details or assistance.

2. History of Injury (Patients Perspective)

a.

Michelle E. Tiu, a 47-year-old female optometrist, reports a history of work-related injuries primarily affecting her shoulders. Her ongoing pain predominantly results from repetitive overhead arm movements and non-ergonomic work environments. Initially, Tiu experienced right shoulder pain due to rearranging workrooms during the COVID-19 pandemic, where she frequently performed non-ergonomic tasks. She has been treated for similar left shoulder issues in the past, which were aggravated by working in various environments requiring forceful overhead equipment manipulation.

Her history indicates frequent overuse and strain from such work conditions, leading to recurrent pain and limitations, managed through physical therapy, ergonomic assessments, medications, and acupuncture. While her left shoulder pain eventually improved with these treatments, recent activities have caused renewed discomfort in her right shoulder, emphasizing the role of ergonomic adaptations in her recovery■4:0†source■.

3. History of Injury (Physicians Perspective)

a.

The patient's history of injuries primarily involves repetitive motion-related issues attributable to her occupation as an optometrist, where she frequently engaged in activities such as reaching and overhead manipulation of equipment. Initially, in 2016, Michelle E. Tiu developed right shoulder pain linked to repetitive work tasks, including reaching to turn off lighting up to 60 times a day. She had undergone ergonomic assessments given her job's demanding physicality■4:0†source■. In subsequent years, a similar pattern of injury emerged in different work environments. By 2017, Tiu experienced left shoulder pain due to extensive overhead pushing and pulling tasks within non-ergonomic conditions■4:4†source■. This recurring theme of shoulder injuries persisted, with new incidents documented as late as 2024, where she developed neck, shoulder, and arm discomfort associated with excessive reaching and non-ergonomic workstations■4:1†source■. Throughout these incidents, Tiu received various treatments such as physical therapy, acupuncture, and ergonomic assessments to manage her symptoms effectively■4:16†source■.

4. Chief Complaints

a.

right shoulder blade:

The patient's chief complaints regarding her right shoulder blade pain, predominantly identified as tendinitis of the right shoulder, are as follows: 1. The patient developed right shoulder pain at work due to repetitive reaching across her workstation to turn off lights up to 60 times during the day. This includes swinging a machine overhead, which aggravates the pain■4:0†source■. 2. She is a 39-year-old right-handed female experiencing moderate to severe constant right shoulder pain and discomfort, which intensifies after about 15 minutes of driving or when reaching with her right arm■4:0†source■. 3. The pain is aching in nature and ranged from 2 to 6 out of 10 in intensity, aggravated by any abduction or movement over chest level, but alleviated by physical therapy and rest■4:2†source■. 4. Additional complaints include shooting pain in the back of her neck and head, which was alleviated at times by acupuncture■4:0†source■■4:4†source■. 5. Another noted chief complaint on a later date was shoulder impingement syndrome of the right shoulder■4:15†source■. 6. The patient also reported mild to moderate right shoulder pain associated with repetitive overhead movements and reaching tasks, often experienced at non-ergonomic workstations due to job-related accommodations during the COVID-19 pandemic■4:14†source■. These symptoms are consistent over several office visits and treatments include NSAIDs, ice application, physical therapy, and various ergonomic adjustments■4:0†source■■4:18†source■.

right scapula:

The chief complaints regarding the right scapula include: 1. **Right Shoulder and Trap Discomfort**: This is described as aching pain on the right posterolateral shoulder, rated at 0-2/10, intermittent. It is aggravated by any abduction, repetitive movement over chest level, and lying on the right shoulder at night. It is alleviated by physical therapy and rest. 2. **Right Shoulder Pain**: Described as moderate to severe, constant pain, which increases after about 15 minutes of driving or when reaching with the right arm. 3. **Bilateral Neck and Shoulder Girdle Pain**: Aching and numbing pain rated at 3-4/10, intermittent. Aggravated by movement, using arms overhead, and pushing and pulling. Alleviated by acupuncture and ibuprofen. 4. **Right Scapalgia**: There is tenderness upon palpation of the right upper trapezius with associated trigger points. These complaints are often associated with repetitive work activities involving reaching across a workstation, which exacerbates the symptoms.

right shoulder:

The patient's chief complaints regarding their right shoulder across various visits are as follows: 1. **Date: 09/15/2016** - Chief Complaint: No specific chief complaint recorded. - Symptoms: Right shoulder pain and discomfort; moderate to severe and constant. The pain developed at work due to repetitive tasks such as reaching to turn off a light multiple times a day and swinging overhead machines. 2. **Date: 09/16/2016** - Chief Complaint: Tendinitis of the right shoulder. - Symptoms: Moderate to severe, constant right shoulder pain. The patient reports increased pain in the right shoulder despite adherence to prescribed work restrictions. Symptoms include decreased range of motion, tenderness, and pain. 3. **Date: 11/16/2017** - Chief Complaint: Shoulder injury. - Symptoms: The patient continued with right shoulder pain which worsens with activities such as overhead and abduction movements. However, improvements were noted post-physical therapy, yoga, and acupuncture, with a transition to self-management. 4. **Date: 12/28/2016** - Chief Complaint: Right shoulder and upper right trapezius pain. - Symptoms: Aching, intermittent pain intensified by chest-level repetitive movements and alleviated by physical therapy and rest. 5. **Date: 07/30/2020** - Chief Complaint: Tendinitis of the right biceps tendon and impingement syndrome of the right shoulder. - Symptoms: Aching and variable right anterior and posterior shoulder pain, aggravated by overhead use and eased by stretching. These reports detail the ongoing management and treatment assessments for the patient's right shoulder pain, which is linked to occupational activities and ergonomic issues.

right elbow:

The patient's chief complaints regarding the right elbow across several visits include: 1. **Elbow Pain and Dysfunction**: - Right elbow pain described as aching, numbing, and burning with a pain rating of 4-5/10, aggravated by any movement, especially using arms overhead, pushing, and pulling. - Ongoing right lateral elbow pain with swelling present and tenderness at the lateral epicondyle. - Difficulty using a joystick at work, pouring orange juice, and washing dishes. - Severe right elbow pain, described as 7/10, with no noted swelling but a bump at the elbow location, particularly upon waking. 2. **Response to Treatments**: - Use of a tennis elbow strap and NSAIDs with limited improvement. - Previous cortisone injections showed mild improvement, and a PRP (Platelet Rich Plasma) injection was later administered. - Noted improvement with acupuncture. 3. **Associated Mechanical Challenges**: - Issues occurred with resting the elbow on newly acquired work equipment contributing to increased pain. - Noted tenderness over the lateral epicondyle with palpable spurring. The complaints are characterized by persistent pain and functional limitations, impacting daily activities and work-related tasks. Various treatments were attempted, demonstrating varied levels of temporary relief and ongoing management steps including advanced imaging and specialist

consultation■4:6†source■4:11†source■.

b.

Right Elbow:

The chief complaints regarding the right elbow for the patient, Michelle E Tiu, include: 1. ****Elbow Pain****: The patient presented multiple times with complaints of right elbow pain, diagnosed as right lateral epicondylitis■4:0†source■. 2. ****Onset and Duration****: Reports indicate ongoing pain in the right lateral elbow with associated symptoms lasting over various periods, such as two months as noted during one of the visits■4:3†source■. 3. ****Associated Symptoms****: The patient describes having trouble with movements requiring elbow use like using a joystick at work, pouring liquids, or washing dishes. This pain was sometimes linked with swelling■4:2†source■. 4. ****Diagnosis Variations****: The pain is generally described in terms of right lateral epicondylitis, commonly known as "tennis elbow," with a mention of joint pain being the primary symptom■4:1†source■4:14†source■. Overall, the issues with the right elbow have led to various treatments and consultations, including considerations for cortisone injections, utilization of elbow straps, and referrals to orthopedics for further management■4:0†source■4:1†source■.

Right Scapula:

The chief complaints regarding the right scapula, specifically related to right shoulder issues, are: 1. ****Incident-related Pain****: The patient reports developing right shoulder pain at work, attributed to repetitive reaching across her workstation to turn off a light multiple times a day, and swinging machinery overhead, which exacerbates the pain■4:0†source■4:7†source■. 2. ****Right Scapulargia and Associated Symptoms****: The patient points to bilateral neck and shoulder girdle pain with aching and numbing sensations, especially uncomfortable with overhead use■4:4†source■4:10†source■. 3. ****Trapezius and Right Scapular Strain****: Complaints include pain in the right upper medial periscapular area extending to the shoulder and neck, worsened by neck movement■4:2†source■4:1†source■. 4. ****Ergonomic Factors and Lifestyle****: There is a recurring issue attributed to the current ergonomics of her work environment, requiring repetitive arm lifting. The situation is noted to improve with physical therapy, acupuncture, and ergonomic modifications■4:8†source■. 5. ****Other Factors****: On another occasion, the right scapula issue is linked to right trapezius strain along with a persistent pain over the deltoid region, noted to benefit from rest, physical therapy, and acupuncture■4:12†source■. These complaints are often aggravated by overhead activities and are occasionally managed or alleviated by medications, physical modifications, or therapy interventions.

5. Claimants Job

a.

The job description for the claimant, Michelle E. Tiu, indicates that she worked as an optometrist for 16 years. Her job responsibilities are stated to include desk work/sedentary work■4:2†source■. Additionally, it is noted that due to the COVID-19 pandemic, she had to work in different non-ergonomic locations, which led to a right shoulder injury. This environment was different from her normal ergonomic setup at her usual workplace■4:2†source■.

b.

The claimant, Michelle E Tiu, works as an optometrist and her job responsibilities primarily involve desk work or sedentary activities. She has been employed in this position for the past 16 years^{■4:0†source■}. Due to certain circumstances, such as working in non-ergonomic work locations during the COVID-19 pandemic, she has had to perform her duties in various exam rooms and offices where standard ergonomic setups were not present. This situation has contributed to a series of musculoskeletal issues^{■4:0†source■}^{■4:1†source■}.

6. Work Status

a.

The most recent work status for the claimant, Michelle E Tiu, is "Modified Activity," which applies both to work and home from 7/31/2024 through 9/5/2024. If modified activity is not accommodated by the employer, she is considered temporarily and totally disabled from her regular work for the designated period^{■4:2†source■}. Additionally, there is a recommendation for a 20-minute stretch break in the morning and a 20-minute stretch break in the afternoon^{■4:2†source■}. Before the current modified activity period, the claimant was evaluated and deemed able to return to work at full capacity on 7/29/2024^{■4:2†source■}.

b.

Patient stopped working on 06/17/2024. The patient was initially placed off work due to incapacitating injury or pain and was subsequently placed on modified activity from 06/26/2024. If the employer cannot accommodate this modified activity, the patient is considered temporarily and totally disabled from their regular work^{■4:0†source■}.

7. Previous Injuries

a.

The patient has not described any previous injuries related to the same body parts. According to the document, "Patient did not have a prior work related injury to the same body part(s). Patient did not have a prior non-work related injury to the same body part(s)."^{■4:11†tmpyms8hkgn.pdf■}.

8. Medical History

a.

The non-surgical medical history of the patient includes the following: - Tendinitis of Right Shoulder - Cervical Myofascial Pain Syndrome - Impingement Syndrome of Left Shoulder - Family History of Breast Cancer - Family History of Ovarian Cancer - Varicella (diagnosed on 01/01/1983)■4:15†source■.

9. Surgical History

a.

The patient's surgical history is as follows: - Orbital blowout fracture s/p motor vehicle accident (MVA), year 1992 - Ankle surgery, year 2006■4:0†source■.

10. Allergies

a.

Here is a list of the patient's allergies: - Dust and grass allergy■4:0†source■. - Vicodin [Acetaminophen-Hydrocodone] causing nausea and/or vomiting■4:2†source■■4:6†source■. - Latex causing lip swelling■4:6†source■. If there are any allergies not listed, such as changes after September 2023, those would need to be noted separately.

11. Current Medications

a.

Here is the list of all the medications the claimant is currently taking: 1. Meloxicam (MOBIC) 15 mg Oral Tab – Take 1 tablet by mouth daily for 10 days, with food. 2. Ibuprofen (MOTRIN) 400 mg Oral Tab – Take 1 tablet by mouth 3 times a day with meals, with food. 3. MetroNIDAZOLE (METROGEL) 0.75% Top Gel – Apply to affected area(s) 2 times a day. 4. Sulfacetamide-Sulfur (AVAR) 10-5% (w/w) Top Clsr – Wash affected area(s) daily as directed. 5. Doxycycline Monohydrate (ADOXA) 50 mg Oral Tab – Take 1 tablet by mouth 2 times a day for acne. 6. PredniSONE (DELTASONE) 10 mg Oral Tab – Take 3 tablets by mouth 2 times a day with meals for 7 days. 7. Diclofenac Sodium (ARTHRITIS PAIN, DICLOFENAC) 1% Top Gel – Apply 2-4 grams to affected area(s) 2 times a day as needed for pain & inflammation (do not exceed 32 grams in 24 hours). 8. Neomycin-Polymyxin B-Dexamethasone (MAXITROL) 3.5 mg/g-10,000 unit/g-0.1% Opht Oint – Apply to lower right eyelid 2 to 3 times a day for 7 days. 9. Tretinoin (RETIN-A) 0.05% Top Crea – Apply to affected area(s) daily at bedtime■4:14†source■.

12. Social History

a.

The medical record indicates the following regarding the patient's use of alcohol, recreational drugs, tobacco, and vaping: 1. **Alcohol Use:** - The patient has stated a response of "never" to alcohol use during proactive care screening. - Additionally, patient history notes an absence of regular alcohol consumption as the weekly alcohol intake is recorded as 0.0 oz. - However, there is a notable exception on 11/06/2016, where the patient presented with nausea and emesis after excessive alcohol intake, indicating a rare instance of alcohol consumption. 2. **Recreational Drug Use:** - The patient has reported no recreational drug use. 3. **Tobacco Use:** - The patient has consistently been recorded as a "never smoker" with no use of smokeless tobacco. 4. **Vaping Use:** - The patient has never used vaping products. It seems the patient generally abstains from alcohol, drugs, tobacco, and vaping, with the exception of a single documented incident of alcohol use.

13. Family history of illness

a.

The family medical history extracted from the provided document is as follows: - **Heart Disease** - Relation: Father - Age of Onset: 60 - Other affected: Maternal uncle - **Cancer** - Type: None reported - **Diabetes** - Relation: Maternal Uncle - Note: 2 maternal uncles affected - **Breast Cancer** - Relation: Maternal Cousin - Age of Onset: age 30s - Note: Different than cousin with ovarian cancer - **Ovarian Cancer** - Relation: Maternal Cousin - Age of Onset: age 30s - Note: Different than cousin with breast cancer - **Childhood Deaths** - Relation: Other - Note: 5 of the patient's mother's siblings.

16. Diagnostic Reports

a.

The document you've provided primarily contains diagnostic imaging reports, none of which include MRI results. Instead, it contains CT and radiology studies. Here is a brief summary of the findings from the available imaging reports in chronological order: 1. **May 13, 2020 - CT Lumbar Spine (No Contrast):** The imaging revealed diffuse mild posterior disc bulges at L3-4, L4-5, and L5-S1. However, there were no significant findings such as spinal canal stenosis, neuroforaminal stenosis, compression fractures, or spondylolisthesis. The lumbar lordosis was normal. 2. **August 28, 2024 - EMG:** This report indicated mild right carpal tunnel syndrome and evidence of chronic right C8 radiculopathy. The available nerve conduction studies showed normal findings except for prolonged median sensory latency across the right wrist. The document did not contain any MRI

results or findings. If studies were ordered but not reported, it's possible that the patient was unable to undergo MRI due to contraindications such as prior surgeries or implants that made MRI infeasible■4:16†source■.

b.

From the search results, it appears there is a consistent CT scan detailed throughout. Here is the summary in chronological order: 1. **Date: 02/17/2020** A CT Lumbar Spine without contrast was initially ordered but discontinued. The clinical diagnosis was lumbar radiculopathy and chronic low back pain, with symptoms including left posterior thigh pain and numbness. The patient could not undergo an MRI due to a facial metal plate■4:16†source■. 2. **Date: 05/11/2020** During a phone encounter, a CT scan was reordered due to persistent back pain. The clinical notes reiterated complaints of chronic low back pain and left posterior thigh pain■4:6†source■. 3. **Date: 05/13/2020** The CT Lumbar Spine was performed, with findings indicating normal spinal alignment and no acute fractures. However, diffuse mild posterior disc bulge was noted at L3-4, L4-5, and L5-S1, without significant spinal or neuroforaminal stenosis■4:0†source■. 4. **Date: 05/18/2020** The diagnostic report from the CT confirmed no acute findings, with conditions matching previous complaints of chronic back conditions. The report was electronically signed, confirming the diagnostic results as final■4:3†source■. This sequence outlines the progression from the initial need for diagnostic imaging due to chronic back issues to the confirmed findings through CT imaging.

c.

Here is a chronological summary of each X-Ray and its diagnostic results: 1. **09/16/2016 - Right Shoulder X-Ray**[†]: This X-ray was performed due to right shoulder pain persisting for a week. The findings showed no acute fracture, with normal alignment and no significant joint disease or soft tissue abnormalities■4:2†source■. 2. **03/20/2017 - Mammogram Screening**[†]: A bilateral digital screening mammogram was conducted, showing extremely dense breast tissue which lowers the sensitivity of mammography. No significant masses, calcifications, or other findings were observed, resulting in a negative impression with no evidence of malignancy■4:10†source■■4:16†source■. 3. **09/17/2019 - Mammogram Screening**[†]: This routine screening showed extremely dense breast tissue in both breasts. Benign calcifications were noted in the right breast, with no other significant findings or interval changes, and a benign impression was recorded■4:3†source■. 4. **10/15/2021 - Mammogram Screening**[†]: Compared to prior exams, there were no significant interval changes, with extremely dense breast tissue observed. No significant masses or calcifications were detected, and the impression was benign with no mammographic evidence of malignancy■4:1†source■. 5. **04/18/2022 - Right Elbow X-Ray**[†]: Requested due to a history of pain, the X-ray showed no acute fracture, with normal alignment and no significant joint disease or soft tissue abnormalities■4:8†source■. 6. **06/09/2023 - Cervical Spine X-Ray**[†]: Conducted due to right-sided cervical radiculopathy. The cervical vertebral bodies had normal height and alignment, but a mild degree of disc space narrowing at C6-C7 and C7-T1 was noted. No fractures or other abnormalities were detected■4:0†source■■4:4†source■. These summaries capture the diagnostic results for each X-ray in the order they were conducted.

d.

1. **May 13, 2020**[†]: The CT lumbar spine imaging revealed mild posterior disc bulges at L3-4, L4-5, and L5-S1. There were no significant spinal canal or neuroforaminal stenoses. The impression concluded no acute findings, and the report was signed by Dr. Jay Kang■4:0†source■. 2. **April 18, 2022**[†]: An X-ray of the right elbow showed no acute fracture or abnormal alignment. Additionally, there were no signs of significant joint disease or soft tissue abnormalities. The report was electronically

signed by Dr. Alfonso Pham. 3. **March 22, 2022**: A chest X-ray study reported clear lungs without pleural effusions and a normal cardiomediastinal silhouette. The findings led to an impression of no acute changes and the report was signed by Dr. Sung Pak. 4. **August 28, 2024**: Nerve conduction and EMG studies identified mild right carpal tunnel syndrome and evidence of chronic right C8 radiculopathy. The conclusion indicated chronic denervation in the right C8 myotome without ongoing denervation.

e.

Here is a summary of the electro diagnostic tests, with findings and conclusions, in chronological order:

1. On February 17, 2020, Michelle E. Tiu underwent a needle electromyography (EMG) for the left lower extremity without related paraspinal areas. The test was stopped upon the patient's request due to intolerance. The diagnosis was lumbar radiculopathy and chronic low back pain, with noted chronic numbness and pain in the left buttocks and posterior proximal left lower extremity.
2. On June 10, 2024, EMG and nerve conduction studies (NCS) were recommended due to ongoing issues with neck pain referring to the arms, specifically the right trapezius strain, neck muscle strain, and muscle spasm. This stemmed from chronic neck pain issues exacerbated by repetitive motions at work.
3. By August 28, 2024, routine nerve conduction studies showed a prolonged right median sensory latency across the wrist, indicating mild right carpal tunnel syndrome (median nerve entrapment at the wrist). Additionally, the needle EMG of the right upper extremity indicated chronic denervation in the right C8 myotome consistent with chronic C8 radiculopathy.

These tests help delineate the nerve issues present, including carpal tunnel syndrome and radiculopathy, which point to chronic nerve compression or damage at specific spinal levels.

17. Doctors Notes

a.

1. **04/10/2015**: On this date, a communication in the form of an electronic image was signed by Eileen M. Perales, a Medical Assistant at Kaiser Permanente, concerning a medical assessment with no available nursing assessment. The encounter was filed at 11:49 AM. There was an absence of calls or messages logged for this particular date. For a following encounter on 05/13/2015, a nursing encounter related to genetics was noted, with Michelle Tiu identified as the caller. The interaction was signed by Eileen, indicating there was no patient response, leading to the closure of the matter by Dr. Hoffman.
2. **04/18/2018**: A telephone encounter was documented by Registered Nurse Rosalind M. Silver at Kaiser Permanente regarding an issue of impingement syndrome of the left shoulder. The interaction was filed at 7:51 AM following the initial contact a few minutes prior, aiming to address Michelle E. Tiu's recurring vaginal infection. This was part of a visit to the Women's Health Center/OB-GYN where the primary complaint of vaginal discharge was assessed. The information was documented along with detailed clinical notes and subsequently electronically signed, marking the official conclusion of the encounter.
3. **09/10/2018**: On this occasion, an orthopedic office visit was documented by Jason Paul Richards, M.D., indicating agreement with the PA's assessment and a treatment plan involving rehabilitation exercises for a rotator cuff issue. Dr. Richards took part in supervising and agreeing with the prescribed exercises aimed at easing shoulder symptoms, including pendulum swings and posterior stretches as part of Michelle Tiu's rehabilitation program. The focus was on gradually increasing exercise intensity while avoiding back pain, and the strategy included several repetitions aimed at enhancing shoulder mobility.
4. **09/11/2018**: Michelle E. Tiu visited the

Family Practice due to a rash on her chest persisting for two to three weeks, with documented refusal of the influenza vaccination. The encounter outlined a Level 3 service with a primary diagnosis of urticaria. Michelle, a 41-year-old female, had her vital signs recorded and assessed by Siara Fulton, L.V.N., and Dr. Smitha Daka, who managed the visit. The comprehensive exam notes highlighted her general physical condition, indicating an absence of medication changes and a clean allergy profile. 5. **05/30/2018**: An office visit with Dr. Michael Young Lee involved an assessment of Michelle E. Tiu's right ankle sprain. This appointment did not include vitals or outpatient prescriptions but administered Triamcinolone Acetonide and planned an X-ray with more than three views. Evaluation included physical examination results showing swelling and a normal range of motion in the right ankle without tenderness. The treatment plan involved rest, ice, elevation, and exercise education, alongside explicit instructions for resolving worsening symptoms or concerns. 6. **04/14/2020**: A communication from Dr. Sung S. Chang in the Family Practice confirmed that Michelle E. Tiu's COVID-19 test was negative. Correspondences also involved access instructions to download work notes via the Kaiser Permanente online portal. The follow-up responses clarified document access steps within the kp.org system, available for viewing and printing. The communication adopted a formal tone, aiming to handle patient inquiries and ensure access to pertinent health records electronically. 7. **07/07/2023**: During an optometry visit documented by Rachel Y. Kim (O.D.), comprehensive evaluations took place, inclusive of visual acuity, refraction, and fundus examinations. The consultation recorded Michelle's prescription changes catering to her refractive errors, including myopia and astigmatism, as well as other eye conditions. Detailed clinical notes highlighted the integrity of Michelle's eye structures and recommended a subsequent eye exam before the prescription's expiration in 2025. A focus was placed on her optometric care plan tailored to maintain ocular health alongside corrective measures.

20. Diagnostic impressions

a.

right shoulder blade:

The diagnostic impressions related to the right shoulder blade, specifically addressing issues with the right shoulder, from the medical records are as follows: 1. Tendinitis of the Right Shoulder: The patient has been diagnosed with tendinitis in the right shoulder, which is identified with the ICD-10-CM code M75.81. This diagnosis has been recurrent in several visits■4:7†source■4:10†source■. 2. Right Shoulder Impingement: There is an indication of right shoulder impingement, though it has shown improvement with persistent symptoms noted on follow-ups■4:17†source■. 3. Right Shoulder Joint Pain: The patient has also been diagnosed with right shoulder joint pain, categorized under the ICD-10-CM code M25.511. This condition is tied to activities experienced during work and is mentioned as a factor causing discomfort■4:2†source■4:19†source■. 4. No Acute Fracture: On imaging studies, no acute fracture was identified, and the joint spaces were maintained. There were no significant joint diseases or soft tissue abnormalities noted in the imaging results from multiple examinations■4:6†source■4:11†source■. The patient's condition was often associated with work-related activities requiring repetitive arm movements, which seemed to exacerbate the shoulder discomfort■4:9†source■.

right scapula:

The diagnostic impressions related to the right scapula from the medical records indicate "Right Scapulargia," which is coded as M89.8X1■4:1†source■. This condition is acknowledged in multiple records pertaining to the patient's injury and assessments related to the right shoulder and scapula regions■4:4†source■4:6†source■.

right shoulder:

The diagnostic impressions for the right shoulder as found in the uploaded medical records are: 1. Tendinitis of Right Shoulder (ICD-10-CM: M75.81)■4:0†source■. 2. Right Shoulder Joint Pain (ICD-10-CM: M25.511)■4:3†source■. These diagnoses appear across multiple entries in the records, confirming a consistent concern with tendinitis in the right shoulder and associated joint pain.

right elbow:

The diagnostic impressions for the right elbow found in the medical records are as follows: 1. **Right Elbow Joint Pain (M25.521)** - This was noted consistently throughout various records and associated with symptoms and diagnostic tests for right elbow pain■4:1†source■. 2. **Right Lateral Epicondylitis (M77.11)** - Also known as "tennis elbow," this condition was identified in some records, often associated with the lack of improvement from conservative treatments like brace usage and NSAIDs■4:4†source■■4:9†source■. 3. **Right Elbow Cubital Tunnel Syndrome Secondary to Subluxing Ulnar Nerve** - This was diagnosed during an orthopedic visit, where physical examination revealed symptoms such as tenderness and spurring over the lateral epicondyle and subluxation of the ulnar nerve causing radiating symptoms■4:10†source■. These diagnostic impressions were derived from clinical histories, physical examinations, and imaging studies documented in the medical records.

Right Elbow:

The diagnostic impressions for the right elbow mentioned in the medical records include: 1. Right Elbow Joint Pain, coded as [M25.521], which was noted multiple times as a significant concern■4:4†source■. 2. No acute fracture identified during imaging, with normal alignment and no significant joint disease or soft tissue abnormality■4:1†source■■4:13†source■. 3. Right Lateral Epicondylitis, coded as [M77.11], frequently diagnosed as a source of pain and treated with various interventions■4:6†source■■4:10†source■. 4. Right Cubital Tunnel Syndrome secondary to subluxing ulnar nerve was also mentioned as a diagnosis, impacting the elbow condition■4:10†source■. These impressions suggest a history of chronic issues and varying conditions associated with the right elbow pain.

Right Scapula:

The diagnostic impression for the Right Scapula in the medical records is documented as "M89.8X1 RIGHT SCAPULALGIA"■4:0†source■.

21. Causation

a.

The patient's injury is related to their work activity, primarily due to non-ergonomic conditions and repetitive movements required by their job. The patient, Michelle E. Tiu, an optometrist, developed right shoulder pain attributed to repetitive reaching across her workstation to turn off lights up to 60 times a day. Additionally, she was required to swing machinery overhead, further exacerbating her shoulder pain■4:0†source■. The ergonomic evaluation at her regular workstation was noted to be normally adequate; however, due to changes in work settings, particularly during the COVID-19 pandemic, she was required to work in non-ergonomic locations. This adaptation contributed significantly to her discomfort, as these stations did not provide the proper ergonomic setup for her tasks■4:4†source■. The clinical evaluations confirm that the symptoms are consistent with work-related activities, noting that several colleagues also experienced similar shoulder issues under the same conditions. The patient's condition, described as right shoulder joint pain (diagnosed as tendinitis, ICD-10 code M75.81), is viewed as likely work-related due to these repetitive and strenuous

activities■4:15†source■4:0†source■. Additionally, it was noted that the patient's primary treatment included rest, ergonomic assessments, and non-steroidal anti-inflammatory drugs (NSAIDs)■4:14†source■. Overall, the medical conclusions suggest that the injury is linked to the repetitive strain and lack of proper ergonomic support at her workplace ■4:9†source■4:12†source■.

b.

The patient's injury appears to be secondary to cumulative trauma at work due to several factors detailed in the medical documents: 1. ****Repetitive and Non-Ergonomic Work Activities****: The patient, Michelle E Tiu, has developed right shoulder pain due to repetitive reaching across her workstation to turn off a light, up to 60 times a day, and swinging a machine overhead. These activities exacerbate her shoulder pain■4:0†source■. Additionally, she reports that, due to the COVID-19 pandemic, she has had to work in various non-ergonomic locations where the setup was not ideal, further contributing to her shoulder strain■4:3†source■4:10†source■. 2. ****Ergonomic Evaluation and Adjustments****: It is stated that several colleagues also experience similar shoulder issues, leading to ergonomic work station evaluations■4:0†source■. The need for ergonomic adjustments highlights a work environment contributing to musculoskeletal strain. There is documentation indicating that changes in the ergonomic setup were planned due to a cluster of similar cases in the optometry department■4:12†source■. 3. ****Work History and Physical Demands****: As an optometrist, Michelle E Tiu performs desk work, overhead reaching, and repetitive upper extremity motions, which are consistent with activities leading to cumulative trauma injuries■4:4†source■4:6†source■. The descriptions suggest that prolonged engagement in these non-ergonomically optimized tasks significantly contributed to the cumulative nature of her injury. 4. ****Medical Evaluations and Opinions****: Medical evaluations have confirmed that her shoulder condition is consistent with work-related activities. The clinical assessments corroborate that her injuries align with her descriptions of work-related strain■4:14†source■. Furthermore, there were no other pre-existing conditions or injuries to the same body part reported prior to this work-related condition■4:11†source■. Overall, the accumulation of repetitive and prolonged physical stresses associated with her job tasks, especially in non-ergonomic environments, have been identified as the primary contributors to Michelle E Tiu's shoulder injuries, as supported by her medical records and workplace evaluations.

22. Impairment

a.

right shoulder blade:

To determine the appropriate impairment rating for a shoulder condition, specifically for the right shoulder blade (scapula), in accordance with the American Medical Association (AMA) 5th Edition Guidelines, we refer to Chapter 16 of the AMA Guides. This chapter outlines the evaluation process for upper extremity impairments, which includes the shoulder. The AMA Guides utilize various methods to assess impairments, such as range of motion, muscle strength, and sensory deficits. However, Table 16-3 and Figures 16-40, 16-43, 16-46, located in this chapter, are specifically used for evaluating shoulder impairments based on range of motion. In the context found in the documents: - A specific mention states there is "no regional impairment based on range of motion of the bilateral shoulder for extension, abduction, adduction, or external rotation" for the right shoulder, and thus no percentage impairment was concluded for these types of movements based on AMA Guides, 5th edition■4:2†source■. - The guides also indicate that tendinitis, fasciitis, or epicondylitis are not typically given a permanent impairment rating unless some other factor must be considered■4:17†source■. For

your case regarding the right shoulder blade, the absence of deficits in range of motion implies no significant permanent impairment rating based on those criteria per the AMA Guides. However, tendinitis or related conditions might not receive a rating unless there are additional complicating factors that justify such. If the condition is episodic or not consistently presenting with sufficient measurable deficits (as supported by medical assessments and guided examination criteria), it might not be rated as a permanent impairment. For a more tailored and exact impairment rating, addressing specific factors such as pain, loss of function, and effect on daily activities, consulting the sections of the AMA Guides might require professional medical assessment and interpretation by a qualified clinician.

right scapula:

I was unable to find specific information in the uploaded documents regarding the impairment rating for the right scapula according to the AMA 5th edition guidelines. To provide a detailed description on how and why a certain impairment rating may be assigned to the right scapula, I suggest referencing the AMA Guides to the Evaluation of Permanent Impairment, 5th Edition directly. The AMA 5th Edition is a standardized guide used by medical professionals to assign a percentage to a person's level of impairment due to injury or illness. This impairment rating serves as a quantifiable metric to help determine the impact of a medical condition on an individual's ability to perform daily activities and work-related tasks. Impairment ratings are usually based on measurable criteria such as range of motion, strength loss, sensory loss, and other definable parameters of physical or functional loss. For specific guidelines on evaluating the scapula, the AMA 5th Edition provides a structured framework within its chapters that delineate ratings based on various factors such as range of motion or loss of function specific to the upper extremities. Typically, the rating is influenced by: 1. ****Range of Motion (ROM):**** Specific angles and limitations in the movement of the scapula can contribute to determining the impairment rating. 2. ****Functional Limitations:**** The extent to which the scapula affects activities of daily living. 3. ****Pain and Loss of Strength:**** These elements can also influence the impairment percentage. If further details are required, consulting a copy of the AMA Guides directly is recommended. If there are specific sections you would like assistance with, feel free to upload relevant pages or sections for review.

right shoulder:

The AMA Guides to the Evaluation of Permanent Impairment, 5th Edition, provide specific criteria for determining impairment ratings for shoulder injuries. For the right shoulder, the impairment is assessed based on the range of motion, strength, and the presence of pain or other limitations. - ****Range of Motion****: The AMA 5th Edition evaluates the range of motion for flexion, abduction, adduction, external rotation, and internal rotation. Each of these movements is compared against standard ranges, and deviations are considered for impairment ratings. In the case I reviewed, there was no regional impairment based on the range of motion for extension, abduction, adduction, or external rotation of the bilateral shoulders. - ****Strength and Sensation****: The evaluation also takes into account muscle strength and sensory examination results. The motor functions of the right shoulder, including the biceps and deltoid, are typically rated at full strength (5/5), and normal sensation indicates no sensory impairment. - ****Pain and Tenderness****: Conditions such as tendinitis or impingement syndrome are considered when evaluating shoulder impairment. Although persistent, these conditions may not necessarily receive a permanent physical impairment rating unless they result in specific functional limitations. - ****No Amputation or Peripheral Nerve System Impairment****: The standards indicate that there was no impairment due to amputation or peripheral nerve system issues in the discussed case. Impairment ratings for the shoulder are expressed as a percentage of either the upper extremity or whole person impairment, with specific tables converting regional impairments to whole person impairments. For instance, a 3% regional upper extremity impairment would correspond to a 2% whole person impairment. This detailed approach ensures an accurate and fair assessment of the degree of impairment, reflecting both the functional limitations and the clinical findings specific to the patient's condition according to the established guidelines.

right elbow:

To assess the impairment rating for the right elbow using the AMA 5th Edition guidelines, we need to reference Chapter 16, which deals with the evaluation of upper extremity impairments. #### Overview of Guidelines for Right Elbow Impairment Rating: 1. **Range of Motion (ROM):** The impairment evaluation for the elbow joint primarily considers the range of motion. Typically, any limitations in flexion, extension, pronation, or supination are assessed using specific figures provided in the AMA Guides, such as Figures 16-1a, 16-1b, and related tables like Table 16-3. 2. **Impairment Rating Process:** - **Figures 16-40, 16-43, 16-46** are used to evaluate the range of motion of the upper extremities, including the shoulder, elbow, wrist, and hand. These figures provide a standard method for measuring and recording the range limitations, deviations, or deficits. - **Table 16-3** often correlates the specific range of motion findings to a percentage of upper extremity impairment, which can then be converted to a whole person impairment percentage. 3. **Conditions Affecting Rating:** - **Tendinitis, Epicondylitis:** According to the guidelines, conditions such as tendinitis or epicondylitis, common causes of elbow impairment, may have a significant presence but do not automatically result in a permanent impairment rating unless they result in a measurable change in the range of motion or other permanent changes. - **No Permanent Impairment Conditions:** If there is no limitation of motion or measurable anatomical disruption, these conditions might not warrant a permanent rating. However, persistent functional deficits can be considered under different criteria if applicable. 4. **Determining Specific Impairment Rating:** - The guides specify that impairments are assessed based on the function and specific physical deficits witnessed during evaluation. If a right elbow impairment results in a significant restriction of motion or functional limitation, a rating may range from 1% to a higher percentage, based on range restrictions and associated functional impact. - For example, a slight limitation in any of the movement ranges of the elbow might result in a regional upper extremity impairment percentage, which is further translated into a whole person impairment percentage based on the conversion standards provided in the guides. 5. **Final Rating Considerations:** - A stable condition with no expected improvement can attract a permanent rating, while a condition anticipated to improve might only receive a temporary rating. In summary, the AMA 5th Edition provides a structured methodology to assess impairment based on anatomical and functional loss. The actual rating for any elbow impairment would significantly depend on the clinical findings related to range of motion deficits and the presence of conditions detailed in the guidelines.

Right Elbow:

To determine the impairment rating for a right elbow according to the AMA Guides, 5th Edition, the guidelines primarily focus on the evaluation of range of motion, strength, and other specific criteria as outlined in Chapter 16 of the guide which addresses the upper extremities. #### Impairment Rating for Right Elbow 1. **Evaluation of Range of Motion:** - The range of motion for the elbow joint is critical in assigning an impairment rating. Specific figures like flexion, extension, pronation, and supination are considered, and any limitations are expressed as percentages of impairment. - **Figures and Tables Used:** Figure(s) 16-40, 16-43, and 16-46 are commonly referenced for determining the percentage based on impairment from the range of motion. 2. **Consideration of Diagnosed Conditions:** - Conditions such as lateral epicondylitis (tennis elbow), while they can be persistent and painful, do not always receive a permanent impairment rating unless other significant factors like neurological or strength deficits are present. - For instance, "Right Lateral Epicondylitis" was noted without additional factors leading directly to a permanent impairment rating. 3. **Conditions like Tendinitis:** - The guidelines suggest that tendinitis, fasciitis, or epicondylitis are not assigned a permanent impairment unless other elements, such as strength deficits or neurological damage, are contributing. 4. **Integration with Other Findings:** - The impairment rating is influenced by additional findings such as grip strength deficit, which if absent, as noted in some evaluations, does not typically add to the impairment percentage. Overall, while the AMA 5th Edition provides detailed frameworks for assessing impairment, each case is subject to clinical findings and comprehensive evaluation of documented conditions against structured guidelines.

Right Scapula:

The impairment rating for the right scapula, as outlined in the AMA Guides to the Evaluation of Permanent Impairment, 5th Edition, is determined based on several factors. The impairment rating process typically involves assessing the range of motion (ROM) and any structural or functional deficits that might result from a particular injury or condition affecting the scapula. 1. ****Range of Motion (ROM) Evaluation****: The AMA guidelines often utilize ROM assessment as a key factor in determining impairment ratings. For the scapula (and shoulder), the range of motion in various planes is considered, such as flexion, extension, abduction, and adduction. The guides provide specific figures and tables to quantify impairment based on limitations in these movements■4:1†source■. 2. ****Associated Conditions****: Conditions such as tendinitis, fasciitis, or epicondylitis involving the scapula region might be considered, but they generally do not receive a permanent impairment rating unless other complicating factors are present. This is because these conditions can be persistent but do not necessarily result in a permanent functional loss■4:1†source■. 3. ****Assessment of Functional Deficit****: The severity of any functional deficits, as well as the overall impact on the individual's ability to perform daily activities, are taken into account. This includes evaluating any pain, strength deficits (such as grip strength if relevant), or other neuromuscular impairments■4:1†source■. 4. ****Conversion Tables****: The AMA 5th Edition provides tables to convert regional impairment ratings into a whole person impairment percentage. This approach helps standardize ratings across different types of injuries and conditions■4:1†source■. The guides are a recognized standard for evaluating the extent of permanent impairment and help ensure consistency and fairness in workers' compensation and disability determinations. It should be noted that any ratings or assessments should be conducted by a qualified healthcare professional skilled in applying these guidelines.

23. Periods Of Total Disability

a.

right shoulder blade:

Based on the document, the average time an American with a right shoulder blade injury would be completely off work before being transitioned to light duty is approximately 8 days. In the case within the document, the patient was off work from 06/17/2024 to 06/25/2024, after which they were transitioned to modified activity or light duty from 06/26/2024 to 07/09/2024■4:0†source■.

right scapula:

For a right scapula injury, the transition to light duty, which involves modified work activities, can vary based on the severity of the injury and response to treatment. In the document reviewed, a patient with a related condition involving the right shoulder was placed on modified activity for approximately 4 weeks after the initial consultation. During this period, the patient was not allowed to reach above the right shoulder or lift/carry/push/pull more than 5 pounds■4:0†source■. Furthermore, after 3-4 weeks of modified work, the patient was gradually transitioned back to usual tasks over the following weeks■4:0†source■. This implies that for a right scapula injury, a patient may be completely off work or on very restricted duties for at least 4 to 8 weeks before light duty modifications are considered. For a more personalized and accurate timeline, specific medical guidelines would need to be followed, which consider the patient's recovery progress, involvement in physical therapy, and other therapeutic interventions like ergonomic assessments■4:1†source■.

right shoulder:

The documents indicate that for a right shoulder injury, such as tendinitis, the period of being off work before transitioning to light duty can vary. Specifically, Michelle E. Tiu was placed off work from 06/17/2024 through 06/25/2024 due to incapacitating pain, as stated in the clinical notes. After this

period, she was put on modified activity at work from 06/26/2024 through 07/09/2024■4:0†source■. Thus, for this particular case, the time off work before transitioning to modified light duty was approximately 8 days (from 06/17/2024 to 06/25/2024). Following this, the patient could work with restrictions, such as needing a 20-minute break for every hour of work during the modified activity period■4:0†source■. This can be considered a typical timeline, but the exact period may vary depending on the specific circumstances of the shoulder injury, treatment response, and workplace accommodations available to the individual.

right elbow:

The documents provided offer examples of cases involving right elbow injuries and the subsequent management of work duties. For instance, a patient was placed off work from 06/17/2024 through 06/25/2024 due to "Incapacitating Injury or Pain." Following this period, the patient was placed on modified activity from 06/26/2024 through 07/09/2024, indicating a transition to light duty could happen within approximately a week of being completely off work, contingent on the employer's ability to accommodate modified duties■4:2†source■. Another case indicates a more extended timeframe, where modified activity began from 07/31/2024 through 09/05/2024, following full-duty clearance on 07/29/2024. Here, the patient was initially off modified activities due to scheduled appointments and cancellations■4:1†source■. In general, the transition to light duty for right elbow injuries can vary but seems to range from a week to several weeks, depending on the severity and the employer's accommodations. The documents emphasize the need for employer accommodation for a safe modified activity work environment■4:0†source■■4:1†source■■4:5†source■.

Right Elbow:

The uploaded document contains information regarding typical work restrictions and modifications following a right elbow or shoulder injury. For an average timeframe of transitioning back to light duty following an injury, one example from the document shows that: - An individual was placed completely off work from 6/17/2024 through 6/25/2024 due to incapacitating injury or pain. - Following the off-work period, the person was transitioned to modified activity at work from 6/26/2024 through 7/9/2024■4:0†source■. Thus, in this specific case, the average timeframe before transitioning to light duty was about 8-9 days off work, with the provision of adjusted work activities thereafter. This can vary substantially depending on the nature and severity of the injury, along with the accommodations an employer can provide.

Right Scapula:

The document indicates that in the case of a right scapula injury, the patient was placed off work for approximately one week, from 06/17/2024 to 06/25/2024. After this period, the patient was transitioned to a modified activity status from 06/26/2024 to 07/09/2024■4:0†source■. If the modified activity could not be accommodated by the employer, the patient was considered temporarily and totally disabled from their regular work for the designated time■4:1†source■. This suggests that the average time off work before transitioning to light duty can be around 8 to 9 days, followed by a period where work is resumed under modified activity conditions■4:1†source■.

24. Future Medical Care

a.

right shoulder blade:

For the management and treatment of the right shoulder blade issues, the following future medical care aligned with the MTUS (Medical Treatment Utilization Schedule) and ODG (Official Disability

Guidelines) has been recommended: 1. **Physical Therapy**: According to the MTUS Chronic Pain Medical Treatment Guidelines, physical therapy is a critical component for improving function, increasing strength, range of motion, and decreasing discomfort. The frequency recommended is 1-2 times a week for 3-6 weeks, totaling around 6 treatments. Physical therapy can help improve activities of daily living (ADLs), reduce medication usage, and decrease work restrictions^{4:1†source}. 2. **Acupuncture**: As per the MTUS Acupuncture Medical Treatment Guidelines, acupuncture can be used as an adjunct to hasten functional recovery, especially when pain medication is reduced or not well tolerated. It's used to reduce pain, inflammation, and muscle spasm, increase blood flow, and improve range of motion. The recommended frequency is 1-2 times a week for 3-6 weeks^{4:0†source}. 3. **Ergonomic Evaluation**: An ergonomic evaluation is often recommended for workplace modifications to reduce aggravating factors and assist with improving function. This aligns with the MTUS guidelines for workplace improvements^{4:1†source}. 4. **Medications**: Continuing the use of NSAIDs, such as Ibuprofen, might be recommended to manage pain, as long as they are well-tolerated by the patient. If necessary, alternative medications like muscle relaxants may also be considered based on the specific symptoms and tolerance of the individual^{4:2†source}. The above treatments are intended to help manage current symptoms and prevent future exacerbations of shoulder issues, favoring a multimodal treatment approach.

right scapula:

The future medical care needed for a right scapula condition, such as scapulalgia or trapezius strain, according to the MTUS (Medical Treatment Utilization Schedule) and ODG (Official Disability Guidelines) includes the following: 1. **Physical Therapy**: Continued physical therapy is recommended for managing right scapulalgia, right trapezius strain, and neck muscle strain. The therapies are typically conducted once a week for six weeks, noting significant benefits such as decreased pain and improved function in activities of daily living (ADLs)^{4:0†source}. 2. **Ergonomic Evaluation**: An ergonomic evaluation is suggested to assess and possibly modify workplace settings to help reduce aggravating factors and improve functional capacity and return-to-work outcomes^{4:1†source}. 3. **Acupuncture**: Per MTUS, acupuncture is an advisable treatment when pain medication is reduced or not tolerated. It can serve as an adjunct to other physical rehabilitation methods or surgical interventions to expedite functional recovery^{4:5†source}^{4:7†source}. 4. **Home Exercise Program**: Continuation and potentially initiation of a home exercise program focusing on scapular setting, strengthening, and managing scapular dyskinesis are important. This program should aim to maintain function, enhance strength, and support the overall treatment goals^{4:3†source}. These treatments are designed not only to address current symptoms but also to prevent potential exacerbations of the condition by strengthening the affected areas and optimizing the patient's ergonomics and daily practices.

right shoulder:

Based on the MTUS and ODG guidelines, the future medical care needed for addressing right shoulder issues, such as tendinitis, includes the following treatments: 1. **Acupuncture**: The MTUS guidelines recommend acupuncture as an option when pain medication is reduced or not tolerated. It serves as an adjunct to physical rehabilitation or surgical intervention to hasten functional recovery. Acupuncture can help reduce pain, inflammation, muscle spasm, and increase range of motion^{4:0†source}. 2. **Physical Therapy**: According to the ODG guidelines, physical therapy can also be part of the treatment plan for tendinitis of the right shoulder. Routine physical therapy focuses on modalities and therapeutic exercises to improve the range of motion and manage symptoms^{4:5†source}^{4:12†source}. 3. **NSAIDs**: The continued use of NSAIDs (non-steroidal anti-inflammatory drugs) is recommended to manage pain and inflammation^{4:1†source}. 4. **Ergonomic Evaluation**: An ergonomic workstation evaluation is suggested to identify necessary workplace modifications that can reduce aggravating factors, thereby preventing or reducing future exacerbations of the condition^{4:5†source}^{4:17†source}. These treatment options align with both the MTUS and ODG guidelines, which aim to address current symptoms while preventing future exacerbations of shoulder pain conditions.

right elbow:

According to the Medical Treatment Utilization Schedule (MTUS) and the Official Disability Guidelines (ODG), the following future medical care may be considered for right elbow issues, such as epicondylitis (often referred to as "tennis elbow"): 1. **Physical Therapy:** - Regular sessions of physical therapy are recommended to improve function, increase strength, range of motion, and flexibility, and to help minimize discomfort. 2. **Acupuncture:** - Acupuncture may be used as an adjunct treatment to reduce pain, promote relaxation, and assist in reducing muscle spasms. 3. **Injections:** - Platelet-Rich Plasma (PRP) injections can be considered for cases, particularly when other treatments like corticosteroid injections have failed. 4. **Ergonomic Evaluations:** - Assessment of the work environment to identify necessary adjustments aimed at reducing exacerbating factors and preventing future aggravations. 5. **Home Exercise Programs:** - Exercises focusing on strengthening and stabilization of the affected area are recommended. 6. **Use of Bracing:** - Wearing braces specifically designed for epicondylitis may help manage symptoms. 7. **Medications:** - Nonsteroidal anti-inflammatory drugs (NSAIDs) may be prescribed to help manage pain. These treatments are considered under guidelines to provide relief from current symptoms and help prevent future exacerbations.

Right Elbow:

According to the Medical Treatment Utilization Schedule (MTUS) and Official Disability Guidelines (ODG) for future medical care regarding the right elbow, here are the potential treatments: 1. **Medications:** - Non-steroidal anti-inflammatory drugs (NSAIDs) for pain management. - Possible use of topical medications to relieve symptoms. 2. **Physical Therapy:** - Physical therapy is recommended with a frequency of 1-2 times a week for 3-6 weeks, totaling approximately 6 treatments. Therapy is intended to improve function, increase strength, range of motion, and flexibility, and help minimize discomfort. 3. **Injection Therapies:** - Consideration of corticosteroid or Platelet-Rich Plasma (PRP) injections for persistent symptoms, especially in cases of conditions like tennis elbow. 4. **Ergonomic Evaluations:** - An ergonomic evaluation may be necessary to assess for workplace modifications to reduce aggravating factors and improve function. 5. **Consultations:** - Access to orthopedic consultations if flare-ups do not subside with listed treatments. 6. **Acupuncture:** - Acupuncture is suggested for pain and spasm management, typically once or twice a week for several weeks. These guidelines aim to provide a comprehensive approach to managing chronic elbow conditions through various standard methods that align with ODG and MTUS recommendations. This approach should help to relieve current symptoms and prevent future exacerbations.

Right Scapula:

Based on the information extracted from the provided document, future medical care recommended for the right scapula in accordance with the Medical Treatment Utilization Schedule (MTUS) includes additional physical therapy. This is recommended to manage right scapulalgia, right trapezius strain, and neck muscle strain. The frequency for this therapy is suggested to be once a week for six weeks, totaling six treatments. This course of treatment aims to reduce pain and improve the function of activities of daily living (ADLs), minimize medication usage, and enhance return to work (RTW) capabilities. The mentioned treatment aligns with the MTUS Chronic Pain Medical Treatment Guidelines in the Physical Medicine Treatment section. Moreover, ergonomic evaluations are also advised to assess workplace modifications that may alleviate aggravating factors and improve function and the ability to return to work. The Official Disability Guidelines (ODG) apparently are not explicitly detailed in this excerpt; hence, you may need to consult a related section in the document that could not be reached with the current search for additional insight. If more detailed ODG guidelines are required, further document sections may need to be examined.

